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Executive Brief | April 2026

Adrabetadex Provider Net Cost Recovery Model Concept Brief

Executive Summary

Adrabetadex has the potential to enter launch with a compelling clinical story in a devastating ultra-rare pediatric disease. Based on current planning assumptions, Beren is preparing for an anticipated August 2026 PDUFA date in infantile-onset Niemann-Pick disease type C, supported by a supplied clinical narrative showing a 71% reduction in mortality risk and a pricing assumption of $39,000 per 900 mg vial administered every two weeks. That translates to approximately **$1.014 million in annual gross drug exposure per patient** before administration, pharmacy handling, denial management, and financing costs. [1] That level of clinical and economic significance creates a clear launch imperative: **centers must be confident they can treat patients without taking unacceptable financial risk.**

The Provider Net Cost Recovery Model gives Beren a practical way to convert Adrabetadex's clinical promise into provider adoption readiness. It shows where reimbursement, coding, pass-through status, 340B eligibility, payer controls, and site-of-care rules support treatment, and where they may create friction. In doing so, the model helps Beren identify which centers are most launch-ready, where field reimbursement support is needed, and what conditions must be in place for providers to administer Adrabetadex sustainably.

Core point

The Provider Net Cost Recovery Model will help ARMs answer the key operational question: after payment, acquisition cost, coding status, payer friction, denial risk, manufacturer support, and payment timing are applied, does a provider make or lose money treating this patient?

Provider Model Value

Adrabetadex has the risk profile of a high-cost, provider-administered specialty therapy. Adoption will depend not only on payer coverage, but on whether the treating center can execute the full reimbursement pathway.

Launch issue	What the provider sees	Why it matters
Acquisition exposure	Approximately $39,000 per vial before payment.	Creates working-capital and inventory risk.
HCPCS uncertainty	Early use may require NOC or temporary coding before a product-specific code.	Manual billing increases documentation burden and denial risk. [2][4]
Payment basis	WAC, AWP, ASP, contractor pricing, or contracted rates may apply depending on site and timing.	Small differences can determine whether treatment is profitable or loss-making. [3]
Payer controls	Prior authorization, site-of-care limits, and specialty-pharmacy sourcing.	Clinical need does not eliminate administrative friction. [11][12]
Medicaid / 340B complexity	EPSDT may support medical necessity, but NDC, state payment, MCO, and duplicate-discount rules still matter.	Coverage does not guarantee collectible payment. [7][8][9][10]

Positioning

The model is not a price calculator, but rather a reimbursement execution model that connects payer strategy, provider activation, field reimbursement, and finance.

How Buy-and-Bill Is Different From EAP

Early Access Program experience can mislead launch planning. In an EAP, providers may not carry the same acquisition, billing, or payment risk they face after commercial launch.

Pathway	Provider economics	Launch implication
EAP	Drug cost and reimbursement exposure may be limited or removed.	Treatment experience does not prove commercial adoption readiness.

Pathway	Provider economics	Launch implication
Commercial buy-and-bill	Provider acquires the drug, bills payer, waits for reimbursement, and bears denial/payment risk.	Viable only if payment, denial rate, and days to payment support a positive net position.
White-bagging / specialty-pharmacy sourcing	Specialty pharmacy bills for the drug; provider may bill only administration.	Drug margin is removed, but operational burden remains. [11]
Site-of-care steering	Payer may restrict where the drug may be administered.	COE strategy must match clinical capability with payable site-of-care rules. [12]

UnitedHealthcare's public sourcing materials for intrathecal administration describe a pathway where the specialty pharmacy bills the payer for the drug and outpatient providers may only seek reimbursement for administration, illustrating how commercial payer rules can remove provider drug economics from the treatment pathway. [11]

What the Model Will Provide and Calculate

The model should calculate provider economics by scenario, payer type, site of care, and channel.

Model domain	Inputs	Output
Product and administration	Dose, WAC, vial size, dosing frequency, procedure assumptions, pharmacy handling, labor, overhead.	Per-dose and annual economics.
Coding and payment	HCPCS status, NOC/C-code/J-code pathway, OPPS pass-through, WAC/AWP/ASP basis, allowed amount.	Expected drug payment and claim risk.
Commercial medical benefit	Prior authorization, white-bagging, site-of-care rules, denial rate, appeal recovery, days to payment.	Buy-and-bill viability.
Medicaid	EPSDT relevance, FFS vs MCO, state PAD methodology, NDC capture, utilization reporting.	Whether medical necessity translates into payment.
340B	Eligibility, estimated acquisition cost, carve-in/out posture, duplicate-discount controls.	Whether the center has a structural acquisition advantage.
Manufacturer support	Acquisition support, bridge logic, appeal support, prompt-pay assumptions.	Break-even support threshold.

Core Formula

The model should use one operating equation across all scenarios:

Expected provider net position

Drug payment + administration revenue + provider-facing support - drug acquisition cost - administration and handling cost - denial/rework cost - payment-delay carrying cost.

Component	Plain-English meaning
Drug payment	What the payer allows for the drug claim.
Administration revenue	Payment for the procedure and related services.
Acquisition cost	What the provider pays for the drug, or avoids paying under white-bagging.
Denial / rework cost	Expected administrative burden and revenue leakage.
Carrying cost	Financing cost while waiting for reimbursement.
Break-even support	Support needed to prevent provider loss.

Coding and Pass-Through Timing

Coding and pass-through should be modeled as separate but related adoption drivers.

Timing step	Fact / planning point	Model implication
PDUFA timing	August 2026 is a client-supplied planning assumption. [1]	Build launch scenarios around early coding uncertainty.
HCPCS Level II cycle	CMS lists drug and biological product application deadlines as the first business day of each quarter: January, April, July, and October. [2]	Test when a product-specific J-code could become available.
HCPCS limitation	CMS states HCPCS coding does not itself determine coverage or payment. [2]	Coding reduces friction, but does not guarantee reimbursement.
C9399 bridge	CMS created C9399 for certain new FDA-approved outpatient drugs without product-specific HCPCS codes, with payment at 95% of AWP as determined by the contractor. [3][4]	Model hospital outpatient bridge payment separately.
Initial sales period	CMS guidance states payment may be up to 103% of WAC when ASP is not yet available. [3]	Use as a Medicare benchmark, not a commercial guarantee.
OPPS pass-through	Pass-through applications are handled through CMS MEARIS; pass-through status is temporary and subject to CMS approval. [5][6]	Treat January 1, 2027 as a planning scenario, not a certainty.

Why 340B Matters

340B should be modeled as an acquisition advantage, not as a guaranteed margin. HRSA describes the 340B ceiling price as based on AMP minus URA, with operational adjustments for package and case-pack pricing. [7][8]

Scenario	Provider position
Non-340B commercial COE	Purchases at or near WAC, depends heavily on payment rate and denial timing.
340B COE	May acquire at a materially lower cost, improving net recovery potential.
Medicaid 340B	Requires carve-in/out and duplicate-discount controls.
White-bagging	Provider does not buy the drug, but also loses drug margin opportunity.

Key implication

Adrabetadex launch may concentrate in 340B-eligible hospital systems because they are more likely to withstand acquisition cost, timing, and payer friction.

Strategic Value for Beren

Decision need	What the model shows	Leadership use
Site prioritization	Which COEs can treat without expected loss.	Focus launch resources.
Provider value story	Whether drug cost, administration, and administrative burden are recoverable.	Support provider education.
Payer friction	Effect of white-bagging, prior authorization, and site-of-care rules.	Shape contracting and field strategy.
Support strategy	Support needed to reach break-even.	Target support instead of over-subsidizing.
MLR clarity	Separation of public facts, internal assumptions, and model outputs.	Reduce review risk.

Recommended Build

The first build should be a disciplined Excel model with a printable executive PDF output tab

Tab	Purpose
Instructions	Define audience, limitations, and scenario use.
Core Assumptions	WAC, dose, payment basis, denial rate, labor, handling, days to payment.
Scenario Selector	Commercial buy-and-bill, white-bagging, Medicaid, 340B COE, pass-through.
Calculation Engine	Locked formulas and source notes.
Executive Output	Per-dose margin, annual exposure, cash burden, break-even result.
Source Appendix	Public source notes and assumption status.

Final Positioning

The Provider Net Cost Recovery Model is a strategic launch tool that translates uncertainty into actionable insight. It makes provider adoption risk visible before launch, allowing Beren to anticipate where Adrabetadex will be financially viable and where barriers may emerge.

It does not compete with the clinical value of Adrabetadex, it enables it. By clearly defining the reimbursement, coding, and economic conditions required for treatment, the model equips Beren to align payer strategy, provider engagement, and field execution. The result is a more predictable, targeted, and scalable path to adoption across hospitals and centers of excellence, without exposing providers to unacceptable financial risk.

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Source Notes

Source notes distinguish public reimbursement authorities, public payer-policy analogues, and client-supplied planning assumptions. Public sources were reviewed on April 28, 2026.

Reference	Use in brief	Source
[1] Beren Therapeutics / Navisync planning assumptions	Client-supplied assumptions for PDUFA timing, clinical narrative, WAC, vial size, dosing frequency, and annual administrations.	Internal planning assumptions supplied for this concept brief.
[2] CMS, HCPCS Level II Coding Procedures	HCPCS drug and biological application deadlines, effective-date cycles, and CMS statement that coding does not itself determine coverage or payment.	https://www.cms.gov/medicare/coding-billing/healthcare-common-procedure-system/level-ii-coding-process
[3] CMS, Part B Drug Payment Limits Overview	Initial sales period WAC logic, ASP timing, AWP, contractor pricing, and OPPS payment for drugs without an assigned HCPCS code.	https://www.cms.gov/files/document/part-b-drug-payment-limits-overview.pdf-0
[4] CMS, C9399 unclassified outpatient drug payment	C9399 logic for certain new FDA-approved hospital outpatient drugs without product-specific HCPCS codes.	https://www.cms.gov/newsroom/press-releases/medicare-pay-unclassified-fda-approved-drugs-administered-outpatient-departments
[5] CMS, OPPS Pass-Through Payment Status page	MEARIS submission pathway for OPPS drug and biological pass-through applications.	https://www.cms.gov/medicare/payment/prospective-payment-systems/hospital-outpatient/pass-through-payment-status-new-technology-ambulatory-payment-classification-apc
[6] CMS, Transitional Pass-Through Drug and Biological Eligibility Process	Eligibility process and planning timing for transitional pass-through status.	https://www.cms.gov/files/document/determine-eligibility-drugs-biologicals-transitional-pass-through-payment-under-hospital-outpatient.pdf
[7] HRSA, 340B ceiling price calculation	340B ceiling price framework based on AMP minus URA.	https://www.hrsa.gov/about/faqs/how-340b-ceiling-price-calculated
[8] HRSA, 340B pricing formulas	Operational 340B package-size and case-pack pricing formulas.	https://340bpricingsubmissions.hrsa.gov/Help/Manufacturer/Pricing%20Formulas/Pricing%20Formulas.htm
[9] Medicaid.gov, EPSDT	Medicaid EPSDT coverage framework for children under age 21.	https://www.medicaid.gov/medicaid/benefits/early-and-periodic-screening-diagnostic-and-treatment
[10] Medicaid.gov, State Drug Utilization Data FAQ	NDC-based utilization reporting logic under the Medicaid Drug Rebate Program.	https://www.medicaid.gov/faq/regarding-state-drug-utilization-data-sdud-there-official-crosswalk-of-national-drug-code-ndc-and-healthcare-common-procedure-coding-system-hcpcs-codes-which-providers-can-use/index.html
[11] UnitedHealthcare, medication sourcing protocol	Public commercial-policy analogue for specialty-pharmacy sourcing and provider administration-only billing.	https://www.uhcprovider.com/content/dam/provider/docs/public/resources/pharmacy/medication-expansion-sourcing-faq.pdf
[12] UnitedHealthcare, Provider Administered Drugs - Site of Care policy	Public commercial-policy analogue for provider-administered drug site-of-care management.	https://www.uhcprovider.com/content/dam/provider/docs/public/policies/index/commercial/provider-administered-drugs-soc-04012026.pdf